Garda Cósta na hÉireann
IRISH COAST GUARD

Treatment Check Cards

TRIDENT
Training and Consultancy

CC-CGU&S-001

ACTIONS	POSSIBLE SYMPTOMS		TIME:
D Danger and Personal Safety MOI: TRAUMA / MEDICAL	Non-Time Critical	Time Critical	
R Assess Response: Alert / Voice / Pain / Unresponsive	A	V P U	AVPU
Update RCC			
(C) Life Threatening Bleed — YES	time tourniquet applied:		CAT TIME
NO	Control with: Direct Pressure (DP) Indirect Pressure (IP) Tourniquet (if trained and competent)		
A Look / Clear / Secure If P or U open airway using: MEDICAL: Head Tilt / Chin Lift TRAUMA: Graduated Head Tilt / Chin Lift	write HT if appropriate:		HT
B Check breathing rate for 10 seconds - and x 6	12, 18 or 24	0,6,30,30+	BR
BREATHING / INEFFECTIVE BREATHING			Begin CPR - CARD 2
TRAUMA Visual Check – Neck / Chest Stabilise injuries – Neck / Chest	OR	MEDICAL Position to ease breathing	
continue			

ASSESSMENT

from above

C

Find pulse and note rate (adult :neck or wrist, child: arm)

Check central cap refill

Consider internal bleed

42- 120	<42 >120	PR
1 or 2 secs	+3 secs	CRT

D

Visually check head

Consider FAST test

FAST -ve	FAST +ve	FAST

E

TRAUMA • Expose and do head to toe • Keep warm • Enquire SAMPLE

MEDICAL• Enquire SAMPLE • Examine as required

• Plan Evac and assistance required • Complete report form

Evaluate Casualty → TIME CRITICAL or NON TIME CRITICAL → Treat and Evacuate See Treatment Cards

Breathing Rate

10 seconds	0	1	2	3	4	5	6	7	8	9	10	11	12
1 minute	0	6	12	18	24	30	36	42	48	54	60	66	72

Pulse Rate

10 seconds	5	6	7	8	9	10	11	12	13	14	15
1 minute	30	36	42	48	54	60	66	72	78	84	90
10 seconds	16	17	18	19	20	21	22	23	24	25	26
1 minute	96	102	108	114	120	126	132	138	144	150	156

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Signs and symptoms

Pain Score

/10

Allergies

Medications

Past relevant medical history

Last oral intake

Events leading to incident

Return to assessment on completion of SAMPLE

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Responders WILL NOT perform Basic Life Support when:

- The scene is too dangerous
- Unable to perform CPR and / or maintain it
- There are injuries incompatible with life (see below)
- A valid DNR form is present
- If appropriate Health Care Professional stops the rescue

Injuries Incompatible with Life:

- Decapitation
- Body cut in half
- Massive skull & brain destruction
- Decomposition or Putrefaction
- Animals have picked at the body
- Rigor Mortis
- Whole body frozen (chest incompressible)
- Submersion for >60 minutes in non-icy water
- Submersion for >90 minutes in icy water
- Incineration
- Pooling hypostasis

(accumulation of blood in the lower parts of the body after death under the influence of gravity)

If you are in any doubt, at any time, you MUST commence BLS.

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TIME CRITICAL: Monitor every 2 minutes
NON TIME CRITICAL: Monitor every 5 minutes

Time	Airway	Response	Breathing Rate	Capillary Refill	Pulse Rate	TC or NTC
	YES	A	12, 18 or 24	1 or 2 secs	>42 <120	
	NO	V P U	0, 6, 30, 30+	3+ secs	<42 >120	

Breathing Rate

10 seconds	0	1	2	3	4	5	6	7	8	9	10	11	12
1 minute	0	6	12	18	24	30	36	42	48	54	60	66	72

Pulse Rate

10 seconds	5	6	7	8	9	10	11	12	13	14	15
1 minute	30	36	42	48	54	60	66	72	78	84	90
10 seconds	16	17	18	19	20	21	22	23	24	25	26
1 minute	96	102	108	114	120	126	132	138	144	150	156

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No Pain	Mild Pain	Moderate Pain		Severe Pain	
0	1 - 3	4 - 6		7 - 10	
					
0	2	4	6	8	10
No Hurt	Hurts Little bit	Hurts Little More	Hurts Even More	Hurts Whole Lot	Hurts Worst

Step 1; Positional Pain Relief

Consider, where appropriate;
immobilisation, elevation, cooling, casualty position

Step 2; Casualty's Own Medication

If available and appropriate, encourage casualty to administer their own pain relief within dosage limits

Step 3; Paracetamol - Mild to moderate pain

USE for pain;

- If indicated in check cards and not contraindicated below

DO NOT use for;

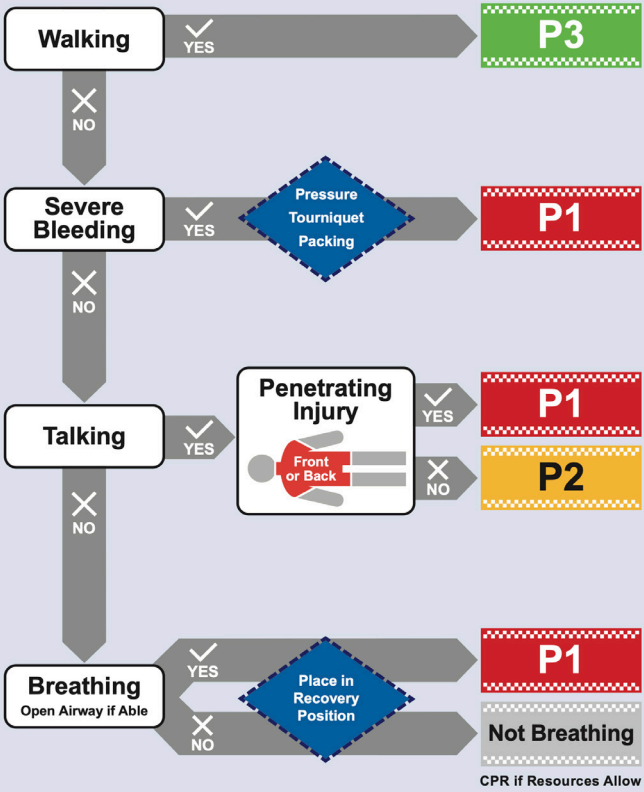
- If casualty is allergic to Paracetamol
- If taken Paracetamol containing medicine in last 6 hours
- If casualty has severe liver failure

Assist ONLY if available and clinical approval given.
Record on medications record card;

Under 12 years;	Oral suspension with dose as per packet
12-15 years;	1x500mg tablet
16 years+;	2x500mg tablet

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Ten Second Triage (TST)



Ten Second Triage (TST)

Ten Second Triage (TST) Tally Chart

P1	P2	P3	Not Breathing
1	1	1	1
2	2	2	2
3	3	3	3
4	4	4	4
5	5	5	5
6	6	6	6
7	7	7	7
8	8	8	8
9	9	9	9
10	10	10	10
11	11	11	11
12	12	12	12
13	13	13	13
14	14	14	14
15	15	15	15
16	16	16	16
17	17	17	17
18	18	18	18
19	19	19	19
20	20	20	20

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Possible signs and symptoms:

Response P or U

(Casualty breathing)

TRAUMA	MEDICAL
CONSIDER the C Spine	Maintain airway using **HEAD TILT / CHIN LIFT**
Maintain airway using **GRADUATED HEAD TILT / CHIN LIFT**	Place in the **RECOVERY POSITION**
Treat traumatic injuries Keep in position found as long as airway is open	**KEEP WARM** with blanket
KEEP WARM with blanket	Monitor and get assistance
Monitor and get assistance	

NOTES

If at any point you cannot maintain the airway in an injured casualty - choose life over limb and use head tilt / chin lift or the recovery position.

Place pregnant casualties on their left side.

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Possible signs and symptoms:

• **Response U** • **Breathing Rate 0 or 6** • **Pale / blue colour**

TIME CRITICAL

ADULT > 16 YEARS
<2 BREATHS IN 10 SECS
OR NO BREATHING - START CPR

Connect **AED** soon as possible and follow **AED** prompts.

OPEN AIRWAY

For adult drowned casualties,

GIVE 5 BREATHS
via pocket mask or face shield

GIVE 30 COMPRESSIONS
at a rate: **100-120** per minute

GIVE 2 BREATHS
via pocket mask or face shield

Continue **30** compressions to **2** breaths, only stopping if shows obvious signs of life

If possible raise casualty's feet.

TIME CRITICAL

CHILD < 16 YEARS
<2 BREATHS IN 10 SECS
OR NO BREATHING - START CPR

OPEN AIRWAY
For Infants keep head in neutral

GIVE 5 BREATHS.
via pocket mask or face shield

Give **CPR**

30 COMPRESSIONS
at a rate: **100-120** per minute

GIVE 2 BREATHS
via pocket mask or face shield

Connect **AED** soon as possible and follow **AED** prompts.

Continue **30** compressions to **2** breaths, only stopping if shows obvious signs of life

NOTES

Chest compression only CPR is acceptable if breaths are not possible
Always attempt CPR unless indicated on the "Conditions Incompatible with Life" Card

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Possible signs and symptoms:

• Breathing Rate 30+ • Cap refill 3+ secs • Pulse >120
• Pale • Cold and clammy • Loss of consciousness

• Keep injured casualties warm and handle gently •

NON CATASTROPHIC	CATASTROPHIC
Sit or lie casualty down Expose and examine wound	Request immediate assistance from RCC
Apply direct finger pressure to site and elevate if possible	Apply immediate direct or indirect pressure
Apply bandage	For limb bleeds use tourniquet if trained and competent
If dressing bleeds through **SLOWLY,** apply additional trauma bandage	Consider **Haemostatic** or normal trauma dressing
If bleeding not controlled, treat as **Catastrophic**	**DO NOT** release tourniquet
If casualty goes unconscious maintain airway **SEE CARD 1**	Note time of tourniquet application on cards / tourniquet / forehead
	If casualty goes unconscious maintain airway **SEE CARD 1**
	Prevent chilling and elevate lower limbs if possible

NOTES

Do not pull out embedded objects.
Pad around objects or open fractures.
Pack holes with haemostatic dressing, then apply direct finger pressure.
DO NOT push haemostatic into Chest, Abdomen or Pelvic cavity.
Apply tourniquet as close to the wound as possible, if bleeding still not controlled apply second tourniquet above the first.
Tourniquets are ONLY to be used by those trained and competent

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Possible signs and symptoms:

• Breathing Rate 30+ • Cap refill 3+ secs • Pulse >120
• Pale • Loss of consciousness
• Cold and clammy • Bruising, discolouration, swelling
• Hardening in the abdomen • Loss of radial pulse

• Keep injured casualties warm and handle gently •

Examine 4 possible locations:
CHEST / ABDOMEN /
PELVIS OR LIMB FRACTURE

Lie casualty HEAD DOWN
Raise feet if possible but not if
leg or pelvis fractures present

Insulate and keep warm

Monitor, reassure and get assistance

NOTES

Be ready to treat unconsciousness..... SEE CARD 1
Be ready to resuscitate..... SEE CARD 2

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Possible signs and symptoms:

- **Difficulty breathing**
- **Wheezing or silent**
- **Loss of consciousness**

Effective Cough? **Yes**

No

Encourage casualty to cough

5 Backblows - after each backblow check to see if obstruction has been relieved

5 Abdominal thrusts (chest thrusts for infant) - after each thrust check to see if obstruction has been relieved

Repeat until resolved or unresponsive

If resolved - **Full Reassessment**

If casualty becomes unresponsive **SEE CARD 2**

Monitor, evacuate if required and hand over to ambulance service

Make casualty aware that if they have received abdominal thrusts they should seek medical attention.

NOTES

Be ready to treat unconsciousness..... **SEE CARD 1**

Be ready to resuscitate..... **SEE CARD 2**

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Possible signs and symptoms:

• Breathing Rate 30+ • Cap refill 3+ secs • Pulse >120
• Pain • Pale and sweaty
• Swelling, deformity, discolouration

• Keep injured casualties warm and handle gently •

CLOSED FRACTURES

Remove watches / jewellery from affected limb

Immobilise fracture using splints or triangular bandages

Insulate and reassure
Monitor and get assistance

OPEN FRACTURES

Control life threatening bleeding, if present

Remove watches / jewellery from affected limb

Apply dressing to wound, pack around bone ends but **DO NOT** push back in

Immobilise fracture using splints or triangular bandages

Insulate and reassure

Monitor, and get assistance

CRUSHED LIMBS

If limb is crushed for >15 mins leave where they are and await help. If crushed for <15 mins remove object and treat any injuries

NOTES

Be ready to treat unconsciousness..... SEE CARD 1
Be ready to resuscitate..... SEE CARD 2

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Possible signs and symptoms:

• Breathing Rate 30+ • Cap refill 3+ secs • Pulse >120 • Pain • Pale and sweaty • Deformity • Wounds

• Keep injured casualties warm and handle gently •

PENETRATING INJURY

Pad around embedded objects **DO NOT PULL OUT**

Seal holes in the chest with dressings, but leave one side per dressing open

If conscious, position casualty in a sitting position

If unconscious, lie casualty on their back and maintain airway with **HEAD TILT / CHIN LIFT**

Apply dressing to any additional wounds and apply pressure

Monitor, reassure and get assistance

BLUNT IMPACT INJURY

If conscious, position casualty in a sitting position

If unconscious, lie casualty on their back and maintain airway with **HEAD TILT / CHIN LIFT**

Monitor, reassure and get assistance

NOTES

Wound size does not equal severity

Be ready to treat unconsciousness:

SEE CARD 1

Prepare to support breathing / CPR

SEE CARD 2

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Possible signs and symptoms:

• Lightheadedness • Nausea • Anxiety
• Sensations of flushing • Tingling or Numbness Arms or Legs
• Visual Disturbance • Feeling of about to faint

• CAUTION: Personal safety is paramount.
If still suspended only rescue if trained to do so •

NOT SUSPENDED	STILL SUSPENDED
Place casualty in a horizontal position as soon as is practically possible	Advise casualty to move legs to encourage blood flow back to the heart
If circulation compromised, remove climbing harness	Elevate lower limbs if possible during rescue
Monitor vital signs and treat as found	Ideally lower down to point of rescue if possible and does not delay rescue
Casualty must be evacuated to appropriate Emergency Department	Once rescued treat as NOT SUSPENDED

NOTES

Be ready to treat unconsciousness..... SEE CARD 1
Be ready to resuscitate..... SEE CARD 2

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Possible signs and symptoms:

• Breathing Rate 30+ • Cap refill 3+ secs • Pain
• Pale and sweaty • Reddening, blistering or charring

Remove from cause

Remove watches / jewellery from affected area where possible

Cool burn for at least **20** minutes, ideally with tepid water

Do not allow casualty to become cold

Dress facial burns if possible, but care that the airway is not impeded. DO NOT use cling film on the face

Dress body or limb burns with cling film
Use patchwork, DO NOT wrap around limb or body

For facial burns suspect airway swelling

Monitor, reassure and get assistance

NOTES

DO NOT burst blisters, or apply adhesive dressings.
DO NOT remove clothing stuck to a burn.

Be ready to treat unconsciousness..... SEE CARD 1

Be ready to resuscitate..... SEE CARD 2

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Possible signs and symptoms:

• Pain • Skin loss • Reddening

• Blistering • Eye irritation

• CAUTION: Consider personal safety •

• Request info from RCC on the chemical involved •

Remove from area of chemical spill
Stop burning process as soon as possible

Remove clothing or jewellery from burnt area
Encourage casualty to cough and blow nose

Brush off powder and irrigate burn with copious amounts of cool / luke warm **running** water for at least 20 minutes,
and up to a maximum of 60 minutes
Irrigate eyes if required

DO NOT DRESS CHEMICAL BURNS WITH CLING FILM

Request immediate assistance
Follow local expert direction
Monitor and reassure

NOTES

DO NOT burst blisters, or apply adhesive dressings.
DO NOT remove clothing stuck to a burn.

Be ready to treat unconsciousness..... SEE CARD 1

Be ready to resuscitate..... SEE CARD 2

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Possible signs and symptoms:

• Breathing Rate 6 or 30+ • Cap refill 3+ secs
• Numbness • Pins and needles • Pain • Incontinence
• Loss of limb movement

Reassure, ask casualty not to move

If standing, ask casualty to lie down

Keep the head still and keep the head and neck in-line with the upper body

Watch for vomiting and clear airway as required

Consider if you have to move **TIME CRITICAL** / **DANGER** or can you stabilise in location **NON TIME CRITICAL**

If moving aim to minimise spinal movement and attempt to keep spine in line

Request immediate assistance . Monitor and reassure

NOTES

Be ready to treat unconsciousness using **graduated Head Tilt** ..... SEE CARD 1

Be ready to resuscitate, using **graduated Head Tilt** ..... SEE CARD 2

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Possible signs and symptoms:

• Breathing Rate 6 or 30+ • Central cap refill 3+ seconds
• Loss of consciousness • Confusion • Irritable
• Combative • Memory loss • Vomiting
• Bruising around the eyes • Bruising behind the ears
• Bleeding from ears or nose • Blood in white of eye
• Flushed skin • Mild general headache • Nausea

Reassure, ask casualty not to move

If standing, ask casualty to lie down

keep the head still and keep the head and neck in-line with the upper body

Watch for vomiting and clear airway as required

Dress head wound keeping head still

Consider if you have to move **TIME CRITICAL** / **DANGER** or can you stabilise in location **NON TIME CRITICAL**

If moving neck aim to minimise spinal movement and attempt to keep spine in line

Request immediate assistance . Monitor and reassure

NOTES

Be ready to treat unconsciousness using **Head Tilt** ..... **SEE CARD 1**

Be ready to resuscitate, using **Head Tilt** ..... **SEE CARD 2**

Casualties have the potential to fit ..... **SEE CARD 13**

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Possible signs and symptoms:

• Response U • Breathing Rate 30+ • Rigid or violent shaking

• CONSIDER REASON FOR FITTING •

TRAUMA

Reassure. Clear the area around the casualty and protect their head

When fit stops, **ASSESS**

If unconscious maintain airway using **HEAD TILT / CHIN LIFT**

SEE CARD 12 HEAD INJURY

MEDICAL

Reassure. Clear the area around the casualty and protect their head

When fit stops, **ASSESS**

If unconscious maintain airway using **HEAD TILT / CHIN LIFT**

Place in recovery position

FEBRILE CONVULSION

Remove clothing and allow to cool

Reassure parents
Prompt parents to administer calpol if appropriate

Casualty must be rapidly evacuated if:

- It is their first fit
- There is repeated fitting
- Fit lasts more than 5 minutes
- Not in a position of safety with a responsible person

Fit Start Time: Fit Stop Time:

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Possible signs and symptoms:

• Pain • Itching • Swelling • Rash • Redness

• Puncture marks • Muscle cramps.

INSECT	JELLYFISH	PORTUGUESE MAN-OF-WAR	WEEVER-FISH	SNAKE
If stinger is embedded remove it by scraping	Scrape off any remaining tentacles, avoid rubbing	Scrape off any remaining tentacles, avoid rubbing	Immerse injured limb in hot water until pain eases	Keep the casualty calm and minimise movement
Wash area with soap and water	If possible apply vinegar or cold sea water	If possible apply hot water	Wash area with soap and water	Treat limb as fractured immobilise with frac straps
Apply cold compress to ease pain and slow absorption	Avoid applying fresh water or cold packs as can increase pain	Avoid applying fresh water as can increase pain	Check and care for puncture wound	Do not use tourniquet or suck wound
Watch for up to 30 minutes for allergic reaction **SEE CARD 17**	Watch for up to 30 minutes for allergic reaction **SEE CARD 17**	Watch for up to 30 minutes for allergic reaction **SEE CARD 17**	Watch for up to 30 minutes for allergic reaction **SEE CARD 17**	Apply pressure dressing work distally increase pressure proximally Seek assistance

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Possible signs and symptoms:

- **Pain - mild indigestion to vice like chest pain**
- **Pain radiation to jaw, neck and arms**
- **Nausea / vomiting**
- **Pale, cold and clammy**

Sit casualty down in comfortable position, loosen clothing and reassure. Avoid lying down unless unconscious

ASCERTAIN CASUALTY HAS NOT TAKEN VIAGRA OR SIMILAR MEDICATION IN LAST 24 HOURS, THEN

If available, encourage casualty to take **their Glyceryl Trinitrate (GTN)** spray or tablets in accordance with directions on pack

CHECK CASUALTY'S AGE, AND THAT THEY DO NOT HAVE BLEEDING DISORDER, ACTIVE GI ULCER OR ARE ALLERGIC

If available, administer 300mg of **ASPIRIN (1 x 300mg)**
Tablet to be swallowed, chewed or dispersed in water

Monitor closely. Be prepared to resuscitate **SEE CARD 2**

If pain is still present, advise second dose of **their GTN**
DO NOT give until at least 5 minutes after the first dose
DO NOT give more than 2 doses in total

Request immediate assistance . Monitor and reassure

NOTES

300mg Aspirin can still be given if the casualty has already taken a daily 75mg dose.

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Possible signs and symptoms:

- **Breathing Rate 30+**
- **Tripod position**
- **Audible wheeze**
- **Difficult and shallow breathing**
- **Unable to complete a sentence**
- **Pale, cold and clammy**
- **Blue if severe**

If Breathing Rate >36, or AVPU is P or U, or unable to speak, understand that condition is potentially IMMEDIATELY LIFE THREATENING

Reassure and remove from trigger if appropriate

Encourage casualty to **SLOW / CONTROL BREATHING RATE**

Sit casualty in **upright** position or **stand** and **lean** forwards

If patient is asthmatic and has their own inhaler, **encourage** them to use it.

Request immediate assistance . Monitor and reassure

NOTES

Take asthma seriously - it can kill

Avoid laying a conscious casualty down as this may aggravate breathing.

Be ready to resuscitate in extreme circumstances ..... SEE CARD 2

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Possible signs and symptoms:

- **Breathing Rate 30+**
- **Tripod position**
- **Audible wheeze**
- **Difficult breathing**
- **Unable to complete a sentence**
- **Red flushed and swollen**
- **Rash**
- **Central cap refill 3+ secs**

Reassure and remove from trigger if possible

Encourage casualty to **SLOW / CONTROL BREATHING RATE**

If casualty is asthmatic and has own inhaler, encourage use of it

Place casualty in position of comfort, **either sitting** or **lying down**

Encourage casualty to use their **ADRENALINE AUTO-INJECTOR (called Epipen, Emerade or Jext)** if they have one

If no improvement and their second adrenaline auto-injector is available, encourage them to use this **5** minutes after the first one

Keep in position of comfort or lying down

Request immediate assistance . Monitor and reassure

NOTES

Do not discontinue first aid procedures even if casualty shows improvement

Send used adrenaline pens to hospital with the casualty.
Be ready to give ventilatory support and be ready to resuscitate in extreme circumstances .....

SEE CARD 2

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Possible signs and symptoms:

• Rapid loss of consciousness AVPU • Aggressive
• Breathing Rate 30+ • Pale, cold, clammy, sweaty
• Mimics drunkenness

NEVER GIVE INSULIN

as if given inappropriately it can kill

CASUALTY UNCONSCIOUS P or U	CASUALTY CONSCIOUS A or V
Maintain airway using **HEAD TILT / CHIN LIFT**	Sit the casualty down and reassure
Place in **RECOVERY POSITION**	Give sugary foods if possible. Ask casualty what they have with them to treat a hypo
KEEP WARM with blanket	Give sweetened drink or sugary foods until casualty is alert on **AVPU**
Monitor, reassure and get assistance	Must be seen by ambulance crew unless fully alert and in a position of safety with a responsible person

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Possible signs and symptoms:

EARLY SIGNS:

• Yawning • Pale and sweaty • Shivering
• Retching • Vomiting

LATE SIGNS:

• Collapse • Incontinence • Lethargic, weak
• Disorientated • Incapacitated

CASUALTY UNCONSCIOUS — P or U

Maintain airway using **HEAD TILT / CHIN LIFT**

Place in **RECOVERY POSITION**

KEEP WARM with blanket

Monitor, reassure and get assistance

CASUALTY CONSCIOUS — A or V

Place the casualty in a well ventilated area but keep warm

Encourage frequent sips of water. Give rehydration solution, isotonic sports drinks, or watered down fruit juice if available

Focus on horizon or encourage sleep

Consider **F.A.S.T.** for Stroke:

- **Face:** Does the casualty's face droop on one side?
- **Arms:** Can the casualty raise both arms and keep them there?
- **Speech:** Is the casualty's speech slurred / altered?
- **Time:** To rapidly evacuate!

The treatment of an unknown illness or Stroke will fall into one of the following three categories:

1 UNCONSCIOUS NOT BREATHING

Give CPR **SEE CARD 2**

2 UNCONSCIOUS BREATHING

SEE CARD 1 - MEDICAL

3 CONSCIOUS

Reassure

Use position of comfort

Monitor and get assistance

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Possible signs and symptoms:

- **Agitation**
- **Cough - may be frothy / bloody phlegm**
- **Breathing Rate 30+**
- **Unable to complete a sentence**
- **Pale and cold**
- **Loss of consciousness AVPU**

CASUALTY UNCONSCIOUS — P or U

- Rapid evacuation from water
- Carry out assessment
- If breathing maintain airway

  SEE CARD 1
- If not breathing start CPR

  SEE CARD 2

CASUALTY CONSCIOUS — A or V

- Rapid evacuation from water and reassure
- Carry out assessment
- If appropriate, remove wet clothing
- Wrap in blankets and insulate from further heat loss
- Monitor, reassure and get assistance
- Advise casualty of the potential for the later complications of drowning

NOTES

Always attempt CPR unless submersion time is more than 90 minutes, or their condition is unequivocally associated with death

Be ready to clear airway - vomiting is usual.

Suspect casualty to also have hypothermia if immersion > 30 minutes.

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Possible signs and symptoms:

COLD:

• <30 mins immersion • Shivering • Conscious • Lucid

HYPOTHERMIA:

• >30 mins immersion • Loss of memory • Foetal position

• Loss of consciousness AVPU • Shivering replaced by spasm

• If immersed > 30mins, remove casualty carefully from the water and lay down •

HYPOTHERMIC: TIME CRITICAL

If not breathing start CPR

SEE CARD 2

If unconscious maintain airway, using **HEAD/TILT, CHIN LIFT**

Place in **RECOVERY POSITION**

Remove outer wet clothing, insulate from further heat loss and cover head

Monitor and get assistance

COLD: NON TIME CRITICAL

Protect from elements and further cooling

If appropriate,remove wet clothing

Insulate from further heat loss and cover head

Give warm sweet drinks

Rational, shivering casualties can be rewarmed by sitting them in a warm shower

NOTES

Supervise rewarming at **ALL** times.

Watch for post rescue collapse, and if it occurs ..... **SEE CARD 1 or 2**

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Possible signs and symptoms:

EXHAUSTION:

• Sweating • Thirst • Weakness • Headache • Nausea • Cramps

HEAT STROKE:

• Hot, dry skin • Confused • Fits • Loss of consciousness AVPU

HEAT STROKE TIME CRITICAL	HEAT EXHAUSTION NON TIME CRITICAL
If not breathing start CPR **SEE CARD 2**	Stop activity and rest in a cool area
If unconscious maintain airway, using **HEAD/TILT, CHIN LIFT**	Remove excess or tight clothing
Place in **RECOVERY POSITION**	Lie the casualty down and raise their legs
Rapidly cool the casualty by any means possible until feels cool to touch	Cool and monitor Should improve within 30 minutes
Start rehydration with sips of water if conscious and appropriate	Start rehydration with sips of water if appropriate
Monitor and rapidly evacuate. Get assistance	If no improvement, advise the casualty to seek medical advice

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Possible signs and symptoms:

• Confused • Agitated • Unpredicatble • Aggressive
• Pacing back and forth • Vacant expression
• Distant and withdrawn • Depressed

• CARE WHEN ASSESSING DANGER (D) and when EXPOSING (E) for potential weapons
• ASSUME NOTHING - there are many medical reasons for the above symptoms

Threatening & unpredictable	Accepting help and no attempt to harm
DO NOT corner or attempt to restrain	**DO NOT** corner or attempt to restrain
Keep under constant observation. Note changes of position and direction of travel	Keep under constant observation.
Advise RCC immediately of any changes	Listen, reassure and explain
Involve other agencies and communicate accordingly	If self-harmed or suicidal must go to hospital for treatment and mental health assessment

NOTES

If casualty attempts to leave do not restrain. The safety of the responders comes first, and the casualty second.

Harmful, agitated or purposefully violent behaviour presents a real hazard. People presenting like this, regardless of the cause, including **TIME CRITICAL** should be managed with extreme caution & care

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NOTES

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Adapted from an original concept by Paul Savage OBE

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ABBREVIATIONS

AED: Automatic External Defibrillator

AVPU: Alert, Voice, Pain, Unresponsive

BR: Breathing Rate

CRT: Capillary Refil Time

FAST: Face, Arms, Speech, Time

HT: Head Tilt / Chin lift

MOI: Mechanism of Injury

NTC: Non Time Critical

PR: Pulse Rate

TC: Time Critical

Contact Numbers:

MRCC Dublin:	01 662 0922
MRSC Valentia:	066 947 6109
MRSC Malin:	074 937 0103

Contact MEDICO through RCC
Contact Poisons / MSDS sheets through RCC

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Medicines Record

Medicine Name : .....

Administration method: .....

Dosage: .....

Time given: .....

Medicine Name : .....

Administration method: .....

Dosage: .....

Time given: .....

Medicine Name : .....

Administration method: .....

Dosage: .....

Time given: .....

Medicines Record

N ame: **A** ge:

T ime of incident:

M echanism of injury:

I njuries:

S igns and Symptoms:

	Initial	Latest
Response Level (AVPU)		
Breathing (per minute)		
Pulse (per minute)		
Cap Refill (seconds)		
FAST (+ve or -ve)		

T reatment given:

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